

RIVERSIDE MULTISPECIALTY HOSPITAL

Department of Endocrinology

Follow-up Consultation Note

Patient Name:	John Doe	Medical Record #:	MRN-2026-001247
Age / Sex:	55 years / Male	Date of Visit:	2026-11-05
Consultant:	Dr. Sarah Smith, MD (Endocrinology)		

REASON FOR FOLLOW-UP

Routine follow-up for management of Type 2 Diabetes Mellitus and recently diagnosed Essential Hypertension.

INTERVAL HISTORY

Mr. John Doe reports that over the past three months he has experienced a significant increase in workload due to the annual tax filing season. He states that long working hours have reduced his ability to exercise regularly, and he admits to relying on restaurant meals and processed foods more frequently than usual. Although he has continued taking his prescribed medications consistently, he acknowledges that dietary adherence has been poor.

Over the past month he has noticed increasing fatigue, occasional excessive thirst, and mildly elevated fasting blood glucose readings at home, typically ranging between 160–190 mg/dL. He denies chest pain, shortness of breath, dizziness, or episodes of hypoglycemia.

CURRENT MEDICATIONS

Medication	Dose	Status
Metformin	500 mg orally twice daily	Continue
Amlodipine	5 mg orally once daily	Continue
Insulin Glargine	10 units subcutaneously at bedtime	New Prescription

PHYSICAL EXAMINATION & VITALS

Parameter	Current Visit	Previous Visit
Blood Pressure	136/84 mmHg	150/92 mmHg
Heart Rate	80 bpm	78 bpm
Respiratory Rate	16 / min	16 / min
Temperature	98.4 °F	98.3 °F
Weight	90 kg (+3 kg)	87 kg
Height	173 cm	173 cm

Parameter	Current Visit	Previous Visit
BMI	30.1 kg/m²	29.1 kg/m²

LABORATORY RESULTS

Test Description	Current Result	Previous Result
HbA1c	8.9%	7.3%
Fasting Plasma Glucose	178 mg/dL	118 mg/dL
Random Blood Glucose	246 mg/dL	160 mg/dL
Serum Creatinine	0.9 mg/dL	0.9 mg/dL
eGFR	>90 mL/min/1.73m²	>90 mL/min/1.73m²
Total Cholesterol	196 mg/dL	204 mg/dL
LDL Cholesterol	126 mg/dL	132 mg/dL

ASSESSMENT

- Type 2 Diabetes Mellitus:** Demonstrates deterioration of glycemic control despite consistent Metformin adherence. Worsening HbA1c from 7.3% to 8.9% is strongly associated with occupational stress, reduced activity, and secondary dietary lapses.
- Essential Hypertension:** Stable and well-controlled on therapeutic Amlodipine regimen (136/84 mmHg).
- Renal Profile:** Renal function remains entirely normal with zero evidence pointing to diabetic nephropathy.
- Weight Status:** Weight gain noted (+3 kg, current BMI 30.1), shifting back above the obesity class line.

TREATMENT PLAN

- Oral Medications:** Continue Metformin 500 mg orally twice daily and Amlodipine 5 mg orally once daily.
- Injectable Therapy:** Initiate Insulin Glargine 10 units subcutaneously at bedtime.
- Clinical Education:** Educate patient on safe insulin injection protocols, rotation of sites, and hypoglycemia symptom mitigation.
- Lifestyle Restructuring:** Order resumption of structured diabetic meal plans and reinstate regular exercise routines (minimum 150 minutes/week).
- Support Resources:** Referral submitted to a Certified Diabetes Educator for reinforced lifestyle counseling.
- Tracking & Evaluation:** Patient to maintain a meticulous home glucose log. Schedule repeat HbA1c evaluation in 3 months.

PATIENT EDUCATION

The patient was counselled regarding the progressive nature of Type 2 Diabetes Mellitus and informed that initiation of basal insulin does not represent treatment failure but rather appropriate escalation of therapy to achieve long-term glycemic control. Insulin injection technique was demonstrated in clinic. The patient successfully performed a supervised self-administration exercise and verbalized understanding of hypoglycemia symptoms and management.

FOLLOW-UP

Return in 8 weeks for insulin dose evaluation. Continue regular diabetes and hypertension monitoring.

Electronically Signed By:

Dr. Sarah Smith, MD
Consultant Endocrinologist
Riverside Multispecialty Hospital